

Stanford University Environmental Health & Safety

Bloodborne Pathogens Institutional Exposure Control Plan

August 2025

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1. Stanford University - Bloodborne Pathogens Exposure Control Plan

The Stanford University Institutional Exposure Control Plan (Institutional ECP) outlines the mechanisms for eliminating or minimizing exposure to bloodborne pathogens (BBP) present in human blood and other potentially infectious materials. Principal Investigators (PIs) and supervisors should refer to the Institutional ECP as a resource for information on institutional exposure controls and regulatory procedures.

1.1 Purpose

“Stanford University makes all reasonable efforts to:

- Protect the health and safety of Stanford University faculty, staff, and students
- Provide safe work practices - academic, research, and administrative - for faculty, staff and students
- Provide information to faculty, staff, and students about health and safety hazards
- Identify and correct health and safety hazards and encourage faculty, staff, and students to report hazards
- Provide information and safeguards for those on campus and in the surrounding community regarding environmental hazards arising from operations at Stanford University”¹

To fulfill this policy and comply with the State of California, Department of Industrial Relations, Division of Occupational Safety and Health (known as “Cal/OSHA”) Bloodborne Pathogens Standard set forth in the *California Code of Regulations, Title 8, Section 5193 (8 CCR 5193)*², this *Bloodborne Pathogens Institutional Exposure Control Plan* (hereafter referred to as “Institutional ECP” or “ECP”) has been developed to minimize personnel exposure to bloodborne pathogens (BBPs) in blood or other potentially infectious materials (OPIM).

Requirements outlined in this ECP are mandatory by the Bloodborne Pathogens Standard where the word “**shall**” is used and are advisory in nature where the word “**should**” is used. Stanford University requirements are noted where the word “**must**” is used.

¹ [Health and Safety: Principles, Responsibilities, and Practices \(Research Policy Handbook, October 2020\)](#)

² [Cal/OSHA Title 8 Section 5193 Bloodborne Pathogens Standard](#)

1.2 Scope

The Institutional ECP covers all Stanford University employees with potential for occupational exposure to human blood and/or Other Potentially Infectious Materials (OPIM). This includes but is not limited to PIs, supervisors, researchers, and service/support staff.

1.2.1 Definitions

“Blood” is defined as human blood, human blood components, and products made from human blood.

“Bloodborne Pathogens” is defined as pathogenic microorganisms that are present in human blood and can cause disease in humans. These pathogens include, but are not limited to, hepatitis B virus (HBV), hepatitis C virus (HCV), and human immunodeficiency virus (HIV).

OPIM are defined as:

The following human body fluids: semen, vaginal secretions, cerebrospinal fluid, synovial fluid, pleural fluid, pericardial fluid, peritoneal fluid, amniotic fluid, saliva in dental procedures, any other body fluid that is visibly contaminated with blood such as saliva or vomitus, and all body fluids in situations where it is difficult or impossible to differentiate between body fluids such as emergency response:

1. Any unfixed tissue or organ (other than intact skin) from a human (living or dead)
2. Any of the following, if known or reasonably likely to contain or be infected with HIV, HBV, or HCV:
 - a. Cell, tissue, or organ cultures from humans or experimental animals
 - b. Blood, organs, or other tissues from experimental animals
 - c. Culture medium or other solutions

Additional definitions related to this Institutional ECP are included in Appendix A.

2. Responsibilities

The following groups have responsibilities under this Institutional ECP.

2.1 Principal Investigators (PIs)/Supervisors

PIs/supervisors are responsible for the health and safety of their employees. Their duties include implementing the Institutional ECP within their areas of operation and managing the overall training and safe work practices. The PIs/supervisors may delegate safety duties but retain responsibility for ensuring they are properly performed.

2.2 Supervised Personnel

Supervised personnel are Stanford University employees who handle, use, or have potential occupational exposure to blood and/or OPIM. Their responsibilities include, but are not limited to:

- Submitting a completed “Hepatitis B Vaccine Declaration” form within 10 working days of initial assignment
- Completing all required training
- Following Institutional ECP-established procedures and safe work practices
- Using appropriate personal protective equipment (PPE)
- Immediately reporting exposure incident, sharps injuries, near-misses, or unsafe conditions to PIs/supervisors and/or Environmental Health and Safety (EH&S)

2.3 Stanford University Occupational Health Center (SUOHC)

The Stanford University Occupational Health Center (SUOHC) is responsible for:

- Managing the Hepatitis B (HBV) Vaccination Program
- Providing post-exposure evaluation, follow-up, and consultation for employees exposed to blood or OPIM
- Maintaining medical records required by Title 8 CCR 5193(h)(1)

2.4 Stanford University Environmental Health & Safety (EH&S)

Stanford University Environmental Health and Safety (EH&S) is responsible for:

- Overseeing Institutional implementation of the Bloodborne Pathogens Standard
- Developing the Institutional ECP and providing tools (e.g., guidance, forms, and templates) to minimize potential occupational exposures to blood or OPIM
- Recommending appropriate engineering controls, administrative controls, and PPE
- Preparing and maintaining the University's Sharps Injury Log
- Performing annual reviews of sharps injuries, preparing the Sharps Injury Annual Report, and disseminating findings as appropriate
- Conducting annual reviews of commercially available safety engineered sharps and new exposure reduction technologies
- Reviewing and updating the Institutional ECP annually

3. Exposure Determination

3.1 Job Classifications with Exposure

Stanford University has determined that employees in the following job classifications may have potential occupational exposure to BBP when performing certain tasks and procedures. This determination is made without regard to the use of PPE. These job classifications are as follows:

- Principal Investigator (PI)
- Staff Scientist/Research Scientist
- Post-doctoral Research Fellow
- Student (undergraduate and graduate) Researcher
- Life Science Research Associate/Technician
- Lab Manager
- Phlebotomist
- Physician
- Registered Nurse
- Nurse Practitioner
- Medical Assistant
- Athletic Trainers/Coaches
- Lifeguards
- Department of Public Safety: Deputy, Sergeant, and Lieutenant
- Department of Land, Buildings & Real Estate: Grounds Keepers, Plumbers, Water Shop Technicians, and Electricians
- Department of Environmental Health & Safety: Emergency Response Personnel
- Anatomical Gift Coordinator
- Embalmer

3.2 Exposure Determination

The following questions are used to determine if an employee is at occupational risk of exposure to BBP.

Does the employee ever:

1. Handle human blood or blood products, such as whole blood, plasma, serum, platelets, or white blood cells?
2. Handle human body fluids such as semen, cerebrospinal fluid, vaginal secretions, joint fluid, pleural fluid, peritoneal fluid, pericardial fluid, or amniotic fluid?
3. Work with animals that are infected with human BBP or perform tasks where such animals are housed?
4. Handle unfixed human tissue or organs, including tissue culture cells and cell lines?
(Tissues and organs soaked in chemical preservatives such as alcohol or formaldehyde are considered "fixed")
5. Handle blood, blood products, body fluids or unfixed tissues or organs (including tissue culture cells and cell lines) of animals infected with bloodborne pathogens?
6. Work with BBP or with preparations, such as liquid solutions or powders, containing BBP?
7. Handle sharp instruments such as knives, needles, scalpels, or scissors which have been used by others working with human blood or OPIM, including human organs, tissue or body fluids, or used by others working with similar body parts and fluids from animals infected with the bloodborne pathogens?
8. Enter areas where other individuals work with human or animal blood, body fluid, tissues or organs which are infected with BBP and perform tasks where these body substances come into contact with the laboratory worker's unbroken skin, broken skin, or mucous membranes?
9. Perform tasks which may potentially result in the worker's exposed skin or mucous membranes coming into contact with human or animal blood, body fluids, organs, or tissues, which are infected with BBP?
10. Provide patient care?
11. Clean clinical areas or equipment?
12. Dispose of regulated waste or soiled laundry?

13. Perform first aid where exposure to human blood or OPIM is possible?

14. Clean up spills of human blood or OPIM?

If the answer to any of the questions above is “YES”, then the employee is considered to be at occupational risk of contracting BBP and must be included in the Bloodborne Pathogens Program.

3.3 Tasks and Procedures

Procedures that may be performed by employees in job classifications with occupational exposure to BBP are as follows:

- Using BBP
- Phlebotomy or venipuncture of humans
- Injections into humans or animals using human specimens
- Other use of needles with human specimens
- Handling human tissue, including preparation, dissection, and cutting
- Pipetting, mixing, or vortexing human blood, fluid, or tissue
- Centrifuging human blood, fluid, or tissue
- Handling tubes or other containers of human blood, fluid, or tissue
- Handling contaminated sharps or other contaminated waste
- Cleaning spills of human blood or other body fluids
- Preparing or handling primary or immortalized human cell lines or cultures
- Clean clinical areas and medical equipment
- Providing patient care including but not limited to collecting specimens including blood, urine, saliva, stool, etc.
- Performing first aid where exposure to human blood or OPIM is possible
- Performing job functions (e.g., law enforcement duties, sewer maintenance, etc.) which may potentially result in the worker’s exposed skin or mucous membranes coming into contact with human or animal blood, body fluids, organs, or tissues, which are infected with BBP

4. Methods of Compliance

4.1 Engineering Controls and Work Practices

Engineering controls and regulated work practices are the primary methods for eliminating or minimizing occupational exposure to blood or OPIM. Accordingly, PIs/supervisors must examine, maintain, and replace engineering controls on a regular basis and evaluate and update work practice controls to ensure their effectiveness. All procedures involving blood or OPIM shall be performed in a manner that minimizes splashing, spraying, spattering, and generation of droplets.

4.1.1 Universal Precautions

The principle of Universal Precautions is an infection control approach that treats all human blood or OPIM (e.g., tissue and body fluids) as if known to be infectious with HBV, HCV, HIV, or other BBPs.

All Stanford University employees shall take this approach at all times when working with human blood or OPIM and follow specific practices and procedures detailed in the [Stanford University Biosafety Manual](#), available online.

4.1.2 Engineering Controls General Requirements

PIs/supervisors shall identify and implement available engineering controls that best eliminate or minimize occupational exposure to blood or OPIM.

Examples of engineering controls include:

- Biosafety cabinets
- Sealed centrifuge rotors
- Mechanical pipetting devices
- Needleless systems
- Sharps with engineered sharps injury protection
- Plasticware
- Puncture-resistant sharps containers

PIs/supervisors are responsible for examining and maintaining engineering controls to ensure their effectiveness on an annual basis. Controls that are not working as intended should be replaced/modified as necessary to ensure safe working conditions.

Engineering controls are evaluated by EH&S safety specialists prior to the commencement of work.

Engineered Sharps Injury Protection means either:

- A physical attribute built into a needle device used for withdrawing body fluids, accessing a vein or artery, or administering medications or other fluids, which effectively reduces the risk of an exposure incident by a mechanism such as barrier creation, blunting, encapsulation, withdrawal or other effective mechanisms
- A physical attribute built into any other type of needle device, or into a non-needle sharp, which effectively reduces the risk of an exposure incident

Guidance on appropriate selection is available from:

- The [International Sharps Injury Prevention Society](#) maintains a list of medical safety devices categorized within their medical application
- The Cal/OSHA publication, [A Best Practices Approach for Reducing Bloodborne Pathogens Exposure](#)
- [OSHA Bloodborne Pathogens and Needlestick Prevention](#)
- [EH&S Biosafety Manual](#) (Safety Engineered and Needleless Sharps, pg. 89)

EH&S is responsible for annually identifying new technologies that eliminate or reduce BBP exposure. This includes documenting the evaluation and implementation of appropriate commercially available needleless systems and safety-engineered sharps as new products arrive on the market. New products may be further evaluated by a group comprised of representatives from EH&S, SUOHC, PIs, researchers, clinical users, and other Stanford University personnel as appropriate

4.1.3 Additional Requirements for Sharps

For withdrawal of body fluids, administration of medication/fluids, and other similar tasks involving potential for exposure incidents, sharps devices shall be selected in the following order of preference:

- Needleless systems. Needleless systems shall be used for:
 - Withdrawal of body fluids after initial venous or arterial access is established
 - Administration of medications or fluids
 - Any other procedure involving the potential for an exposure incident for which a needleless system is available as an alternative to the use of needle devices
- If needleless systems are not used, needles with engineered sharp injury protection shall be used for:
 - Withdrawal of body fluids
 - Accessing a vein or artery
 - Administration of medications or fluids
 - Any other procedure involving the potential for an exposure incident for which a needle device with engineered sharps injury protection is available

Sharps without engineered sharps injury protection (“non-safety sharps”). Non-safety sharps shall be selected only if one of the following cases apply:

- Safety sharps are not available in the marketplace
- For patient care, a licensed healthcare professional directly involved in a patient’s care determines and documents that use of an engineering control will jeopardize a patient's safety or the success of a medical, dental or nursing procedure involving the patient. Healthcare providers at Stanford University are responsible for documenting the determination of non-safety sharps usage, if any. A documentation template is available in Appendix B
- “Objective Product Evaluation Criteria” can demonstrate that an engineering control is not more effective in preventing an exposure incident than an alternative already in use. The basis for this determination may include, but is not limited to, studies providing data on the device’s performance and evaluations made by research entities that have no economic relationship with manufacturers
- Reasonably specific and reliable safety performance information is not available, and the PI/supervisor is actively determining by means of objective product evaluation criteria whether use of the engineering control will reduce the risk of exposure incidents in the lab/work area. It is generally sufficient for PIs/Supervisors to rely on

peer organizations, academic studies, and professional journals to track currently available information on devices. It may be appropriate for large departments to make more direct efforts to evaluate devices

4.2 Work Practice Controls

PIs/supervisors are responsible for implementing and regularly evaluating work practices to minimize potential exposure and ensure their effectiveness.

Handling of sharps (e.g., needles, blades, and broken glass) presents a risk of occupational exposure to blood or OPIM. To minimize this risk, the use of sharps requires appropriate care and adherence to safety guidelines.

4.2.1 Handling Contaminated Sharps

All procedures involving contaminated sharps shall incorporate safe handling practices that minimize risks of sharps injuries. These safe handling practices include, but are not limited to:

- Procedures involving the use of sharps in connection with patient care, such as withdrawing body fluids, accessing a vein or artery, or administering vaccines, medications or fluids, shall be performed using effective patient-handling techniques and other methods designed to minimize the risk of a sharps injury
- Place contaminated sharps in a hard-walled sharps waste container immediately or as soon as possible after use
- Sharps containers shall be:
 - Hard-walled, puncture resistant, leak-proof, and labeled with the biohazard symbol as indicated in section 5.1.4.3
 - Easily accessible to personnel and located as close as feasible to the immediate areas where sharps are used or can be reasonably anticipated to be found
 - Maintained upright throughout use, where feasible
 - Replaced once filled to 2/3 fill line, to prevent overfilling

4.2.2 Prohibited Practices

- Shearing or breaking contaminated needles and other contaminated sharps is prohibited
- Contaminated sharps shall not be bent, recapped, or removed from devices
 - Contaminated sharps may be bent, recapped or removed from device only if: a) PI/supervisor can demonstrate that no alternative is feasible or that such action is required by a specific medical or dental procedure; and b) The procedure is performed using a mechanical device or a one-hand technique
- One-hand technique means a procedure wherein the needle or a reusable syringe is capped in a sterile manner during use. The technique requires the use of only the hand holding the syringe so that the free hand is not exposed to the uncapped needle
- Sharps that are contaminated with blood or OPIM shall not be stored or processed in a manner that requires employees to reach by hand into the containers where these sharps have been placed
- Disposable sharps shall not be reused
- Broken glassware which may be contaminated shall not be picked up directly with hands. It shall be cleaned up using mechanical means, such as a brush and dustpan, tongs, or forceps
- Sharps containers shall not be opened, emptied, or cleaned manually or in any other manner which would expose employees to the risk of sharps injury
- Eating, drinking, smoking, applying cosmetics or lip balm, and handling contact lenses is prohibited in work areas that have potential exposure to blood, blood products, or OPIM
- Food and drink shall not be kept in refrigerators, microwaves, cabinets, countertops, or other areas where blood, blood products, or OPIM may be present
- Personnel shall never perform mouth pipetting

4.2.3 Sharps Waste Containers

Sharps that are used in areas where the BBP standard applies, regardless of whether they are contaminated or not, shall be disposed of in containers that are:

- Affixed with Biohazard Labels

- Rigid
- Puncture resistant
- Leakproof on the sides and bottom
- Portable
- Closeable
- Sealable
- Incapable of being reopened easily
- Non-reusable
 - Waste that involves a mixture of regulated and unregulated waste (e.g., hazardous chemical waste, radioactive waste) may require additional guidance found in the [Stanford University Biosafety Manual](#), Section 11, “Waste & Decontamination”

4.2.4 Handwashing

Handwashing facilities must be readily available to employees. Employees shall wash their hands with soap and running water immediately upon removal of gloves or other PPE.

Employees shall wash hands and any other skin with soap and water, or flush mucous membranes with water immediately or as soon as possible following contact of such body areas with blood or OPIM.

If handwashing facilities are not immediately available, employees shall use an appropriate antiseptic hand cleanser in conjunction with clean cloth/paper towels or antiseptic towelettes and then wash their hands with soap and running water as soon as feasible.

4.2.5 Handling Specimens of Blood or OPIM

Universal Precautions must be practiced in the handling of all specimens. Specimens of blood or OPIM shall be placed in biohazard-labeled containers that prevent leakage during collection, handling, processing, storage, transport, and/or shipping.

The specimen container shall be placed in a biohazard-labeled secondary (outer) container if:

- Contamination of the primary (inner) container occurs
- The specimen can puncture the primary container
- Leakage from the primary container may occur

4.2.5 Cleaning and Decontaminating Work Areas

- PIs/Supervisors are responsible for ensuring that their workplaces shall be maintained in a clean and sanitary condition
- Work surfaces shall be cleaned and decontaminated with an appropriate disinfectant immediately or as soon as feasible when:
 - A surface becomes, or may have become contaminated
 - There is a spill of blood or OPIM
 - Work procedures are completed
- All receptacles intended for reuse which have a reasonable likelihood for becoming contaminated with blood or OPIM shall be inspected and decontaminated after use and immediately or as feasible upon visible contamination
- Any protective coverings, such as plastic wrap, aluminum foil, or impervious absorbent paper used to cover equipment and environmental surfaces shall be removed and replaced as soon as feasible when they become overtly contaminated, or at the end of the workday if they may have become contaminated
- Each PI/supervisor of a worksite shall determine, implement, and maintain appropriate methods and written schedules for cleaning and decontamination based upon the location within the workplace, type of surface, type of potential contamination present, and tasks or procedures performed in the area
- Resources available for determining the appropriate decontamination methods include:
 - [Stanford University Biosafety Manual](#), Section 11, “Waste & Decontamination”
 - [EPA-registered disinfectants](#)
 - [EH&S Contact Us form](#)

4.2.6 Shipping

For shipping of blood or OPIM, all safe handling and labeling requirements noted in the Institutional ECP shall be followed. Employees involved with shipping blood, OPIM, or other biological agents must also:

- Follow guidelines in [Stanford University Biosafety Manual](#), Section 10,

“Transportation”

- Complete the STARS online training module “EHS-PROG-2700 Shipping Dangerous Biological Goods or Dry Ice” available online at axess.stanford.edu

4.2.6.1 Servicing Contaminated Equipment

Before servicing or shipping, all equipment which may have become contaminated with blood or OPIM shall be examined and shall be decontaminated as necessary, unless the PI/supervisor can demonstrate that decontamination of such equipment or portions of such equipment is not feasible or will interfere with a manufacturer’s ability to evaluate failure of the device.

Equipment that cannot be decontaminated prior to servicing shall be labeled in accordance with Cal/OSHA standards.

4.2.6.2 Waste Disposal

Employees must follow the “[Medical and Biohazardous Waste Disposal Guidelines](#)” and Section 11, “Waste & Decontamination” of the [Stanford University Biosafety Manual](#).

Further information is available from on the [EH&S website](#).

4.3 Personal Protective Equipment (PPE)

PIs/Supervisors hold the primary responsibility for implementing the PPE Program in their work areas. This includes:

- Ensuring that workplace hazards have been evaluated
- Providing appropriate PPE
- Guaranteeing that employees received necessary training

Stanford University provides the [Laboratory PPE Assessment Tool](#) to facilitate this assessment.

The PPE user is responsible for:

- Wearing PPE as required per the PPE Assessment Tool
- Attending PPE training sessions
- Cleaning and maintaining PPE as trained
- Informing the supervisor of the need to repair or replace PPE.

4.3.1 Provision

PIs/Supervisors are responsible for determining, documenting, and providing the specific PPE requirements for their employees based on an assessment of anticipated exposure to blood or OPIM using the [Laboratory PPE Assessment Tool](#).

Employees will be provided with appropriate PPE at no cost for tasks with potential exposure. This includes but is not limited to:

- Gloves
- Gowns
- Laboratory coats
- Face shields or masks
- Eye protection

PPE will be considered appropriate only if it prevents blood or OPIM from passing through or reaching the employee's work clothes, underlying clothing, skin, eyes, mouth, and other mucous membranes under normal conditions of use and for the duration of the task.

4.3.2 Use

PIs/Supervisors shall ensure employees use appropriate PPE for their specific tasks. Minimum required PPE includes a lab coat, gloves, and safety glasses. Additional PPE may be required if a risk assessment deems it necessary.

Under extraordinary circumstances, employees may decline to use PPE temporarily if, in their professional judgment, it would have prevented the delivery of health care or public safety services or would increase a hazard to themselves or a co-worker. Such instances require alternative administrative and engineering controls to minimize exposure risks. Additionally, the circumstances will be investigated and documented to determine prevention of such occurrences in the future.

4.3.3 Accessibility

PIs/Supervisors shall ensure that appropriate PPE in the correct sizes is readily accessible at the worksite or is issued to employees. Hypoallergic gloves, glove liners, powder-free gloves, or other similar alternatives shall be readily available to those employees who are allergic to the gloves normally provided.

4.3.4 Removal, Cleaning, Laundering, and Disposal

PPE shall be inspected, cleaned, and replaced as needed. Any garment penetrated by blood or OPIM or visibly soiled shall be removed as soon as feasible.

All PPE shall be removed before leaving the containment boundary of the work area. Taking PPE outside of work areas (e.g., wearing PPE in hallways, offices, and other common areas) or offsite (e.g., taking a laboratory coat home to clean) is prohibited. Removed PPE shall be placed in a designated area or container for storage, laundering, decontamination, or disposal.

Disposable PPE worn while working with blood or OPIM shall be treated as regulated waste. It should be changed immediately upon visible contamination or at least every two weeks.

Reusable PPE (e.g., laboratory coats, uniforms, and other garments) potentially contaminated with blood or OPIM shall be either immediately placed in nonpermeable bags affixed with biohazard symbols or disposed of as regulated waste.

4.3.5 Gloves

Gloves shall be worn when working with blood and/or OPIM, when handling or touching contaminated items or surfaces, and when performing vascular access procedures. The following guidelines should be followed:

- Disposable gloves shall not be washed or decontaminated for re-use
- Glove recycling is strictly prohibited for gloves used with blood or OPIM
- Utility gloves may be decontaminated for re-use if the integrity of the glove is not compromised. They must be discarded if cracked, peeling, torn, punctured, or otherwise compromised
- Employees must wash their hands immediately after glove removal

4.3.6 Masks, Eye Protection, Face Shields, and Respirators

Eye protection must be worn when working with blood and/or OPIM, or when handling or touching contaminated items or surfaces.

Masks in combination with eye protection devices (e.g., goggles, glasses with solid side shields, or chin-length face shields), shall be worn whenever splashes, spray, spatter, or droplets of blood or OPIM will be generated.

If respiratory protection is used (beyond a surgical mask), all procedures will be conducted in accordance with the [Stanford University Respiratory Protection Program](#).

4.3.7 Gowns, Aprons, and Other Protective Body Clothing

Appropriate protective clothing (e.g., gowns, aprons, lab coats, clinical jackets or similar outer garments) shall be worn based on the task and anticipated degree of exposure.

Surgical caps or hoods and/or shoe covers or boots shall be worn when gross contamination can be reasonably anticipated.

4.4 HBV, HCV, and HIV Research Operation Requirements

4.4.1 Workplace Controls and Practices

In addition to the general engineering and work practice controls outlined in this Institutional ECP, all research operations involving HBV, HCV, and HIV shall adhere to these measures and all applicable provisions of [Stanford University Biosafety Manual](#).

4.4.1.1 Access

Laboratory doors must remain closed during work with blood or OPIM involving HIV, HBV, or HCV. Access to the workplace must be limited to authorized individuals.

4.4.1.2 Work Practices and Procedures

PIs/Supervisors are responsible for preparing, implementing, reviewing, and updating written biosafety procedures for their work site, as approved by the Institutional Biosafety Committee (IBC). Researchers are advised of the potential hazards and are required to be familiar with these site-specific protocols. Additionally, the following guidelines should be followed:

- Researchers must take special care to avoid skin contact with OPIM and wear gloves when handling infected animals
- Laboratory coats, gowns, smocks, uniforms, or other appropriate protective clothing shall be used in the work area and animal rooms
- Use hypodermic needles and syringes only for parenteral injection and aspiration of fluids from animals and diaphragm bottles
- Only needle-locking syringes or disposable syringe-needle units (i.e., the needle is

integral to the syringe) shall be used for the injection or aspiration of OPIM

4.4.1.3 Engineering Controls

PIs/Supervisors shall identify and implement available engineering controls that best eliminate or minimize occupational exposure.

These engineering controls must include:

- Activities are conducted in biological safety cabinets or within other physical containment devices. No work with blood or OPIM is conducted on the open bench
- Vacuum lines are equipped with liquid disinfectant traps and HEPA filters (or filters of equivalent or superior efficiency) and are checked routinely, and maintained as necessary
- Certified biological safety cabinets (Class I, II, or III) or other appropriate combinations of personal protection or physical containment devices, such as special protective clothing, respirators, centrifuge safety cups, sealed centrifuge rotors, and containment caging for animals, shall be used for all activities with OPIM that pose a threat of exposure to droplets, splashes, spills, or aerosols
- Biological safety cabinets shall be certified that they meet manufacturer's specifications when installed, whenever they are moved, and at least annually
- Eye and handwashing facilities must be readily available within the work area

4.4.1.4 Decontamination Practices

The following decontamination practices must be followed:

- Protective clothing is not worn outside of the work area, and it is decontaminated before being laundered
- Contaminated materials are placed in durable, leak proof, labeled or color-coded containers that are closed before being removed from the work area for decontamination
- All spills are immediately contained and cleaned up by appropriate professional staff or others properly trained and equipped to work with potentially concentrated infectious materials
- Before disposal, all waste from work areas and from animal rooms is treated in

accordance with the Stanford University medical waste management plan

4.4.1.5 Experience, Proficiency, and Training

Before working with HBV, HCV and/or HIV or OPIM, PIs/supervisors shall ensure that researchers have prior experience in handling human pathogens or tissue cultures and demonstrate proficiency in standard microbiological practices and techniques and in the practices and operations specific to the research/work area.

If no prior experience exists, PIs/supervisors shall provide training. Initial tasks must not involve handling infectious agents and work must progress in complexity only as techniques are learned and proficiency is demonstrated. Researchers may only handle infectious agents after demonstrating full proficiency.

4.5 Hazard Communication to Employees

4.5.1 Biohazard Labels and Signs

- Biohazard labels shall either be an integral part of the container or shall be affixed as close as feasible to the container in a manner that prevents their loss or unintentional removal
- Red bags or red containers may be substituted for labels **except for sharps containers or regulated waste red bags**
- Bags used to contain regulated waste shall be color-coded red and shall be labeled
- Chemotherapy, pathology and other regulated waste have specific labeling requirements under the *California Medical Waste Management Act (Health and Safety Code Sections 118275 through 118320)*. PIs/Supervisors must contact the [EH&S Environmental Protection Program](#) if they anticipate generating these types of waste in the work area. Further information about regulated waste can be found in the [Stanford University Biosafety Section 11 "Waste"](#).

4.5.1.1 Items Requiring Labels

The following items must have biohazard labels:

- Sharps waste containers
- Containers for handling, storing, or transporting specimens containing blood or OPIM

- Containers of regulated waste
- Refrigerators and freezers which may contain blood or OPIM
- Cryotanks which may contain blood or OPIM
- Bags containing PPE to be laundered
- Equipment that is used with blood or OPIM
- Previously contaminated equipment than has not been decontaminated

The following do not require biohazard labeling:

- Containers of blood or OPIM labeled as to their contents and released for transfusion or other clinical use
- Decontaminated regulated waste
- Individual containers of blood or OPIM that are placed in a labeled container during storage, transport, shipment or disposal

4.5.1.2 Label Specifications

Biohazard warning labels shall:

- Include appropriate universal biohazard symbols similar to the symbol to the right and the word “BIOHAZARD”
- Be fluorescent orange or orange/red with lettering and symbols in contrasting color



Visit the [EH&S Safety Store](#) to request labels or contact the [EH&S Biosafety](#) for further information on applicable labeling requirements.

4.5.1.3 Biohazard Area Signs

Biohazard warning signs must be posted on access doors to workplaces which contain blood or OPIM, and all work areas for HIV, HBV, or HCV research laboratories.

Biohazard signs shall:

- Include the Universal biohazard symbol

- List the name of the infectious agent(s) present
- Indicate special requirements for entering the workplace
- Provide the name and telephone number of PI/supervisor for the workplace
- Contact [EH&S Biosafety](#) to obtain biohazard signs

4.6 Training

4.6.1 Online Training Modules

All employees with potential for occupational exposure to blood or OPIM must complete the following training modules (available through axess.stanford.edu) upon initial assignment:

- a. EHS-1500: Biosafety
- b. EHS-1600: Bloodborne Pathogens

These employees must also complete the following module annually thereafter:

- a. EHS-1601: Bloodborne Pathogens

PIs/supervisors are responsible for the health and safety of their employees, including ensuring these training requirements are met.

4.6.2 Workplace-Specific Operations & Safe Practices

PIs/Supervisors shall ensure that supervised employees receive workplace-specific training:

- Before beginning work with potential exposure to blood or OPIM annually thereafter
- When changes affect occupational exposure such as:
 - Introduction of new engineering, administrative, or work practice controls
 - Modification of tasks or procedures
 - Institution of new tasks or procedures

5. Hepatitis B (HBV) Vaccination

The HBV vaccination series is available to employees who may have occupational exposure. The vaccine is offered free of charge within 10 working days of initial assignment. If the employee has previously received the complete HBV vaccination series, antibody testing has revealed that the employee is immune, the vaccine is contraindicated for medical reasons, or the employee elects not to take the HBV vaccine, the employee shall sign the “[Stanford University Hepatitis B Vaccine Declaration Form](#)” (Appendix C).

SUOHC is the administrator of the HBV vaccination program and shall ensure that all medical evaluations and procedures, including the HBV vaccination series and post-exposure evaluation and follow-up, including prophylaxis, are:

- Made available at no cost to the employees
- Made available to the employee at a reasonable time and place
- Performed by or under the supervision of a licensed physician or by or under the supervision of another licensed healthcare professional
- Provided according to recommendations of the U.S. Public Health Service current at the time these evaluations and procedures take place

Employees with the potential for occupational exposure to blood or OPIM shall:

- Review the “[Occupational Exposure to Hepatitis B Virus](#)”
- Submit a completed HBV declaration form within 10 working days of initial assignment

While Stanford University encourages employees to be vaccinated, accepting vaccination is not a condition of employment. Employees who decline to take the vaccine will be required to sign the Hepatitis B Vaccine Declaration Form. However, employees who refuse the initial vaccine may change their decision and receive the vaccine at any time for as long as they continue to have occupational exposure.

If a routine booster dose(s) of hepatitis B vaccine is recommended by the U.S. Public Health Service at a future date, such booster dose(s) shall be made available.

PI/supervisors shall ensure that employees electing to receive the HBV vaccine are provided sufficient time during normal business hours to obtain the vaccination from SUOHC.

Additional information regarding the HBV vaccine and occupational exposure to the virus is

available through [SUOHC](#).

5.1 Exceptions for First Aid Providers

First aid providers are not required to be offered pre-exposure HBV vaccine if rendering first aid is not the primary job assignment of the employee. Any first aid rendered is only as a collateral duty responding solely to injuries resulting from workplace incidents, generally at a location where the incident occurred. ***However, this exception does not apply to designated first aid providers who render assistance on a regular basis such as public safety employees who are expected to render first aid in the course of their work and locations where injured employees routinely go for such assistance.***

Additional requirements for first aid incidents:

- HBV vaccine shall be made available to all unvaccinated first aid providers who have rendered assistance in any situation involving the presence of blood or OPIM (regardless of whether an actual exposure incident occurred)
- Post-exposure evaluation, prophylaxis, and follow-ups shall be available for those employees who experience exposure incidents

6. Exposure Incidents

6.1 Emergency Procedures

In the event of an exposure incident involving blood or OPIM, the following actions must be taken immediately:

1. Initiate first aid (as appropriate) in the workplace:
 - a. Wash contaminated skin or wounds thoroughly for fifteen (15) minutes using soap and running water
 - b. Irrigate contaminated eyes and mucous membranes for fifteen (15) minutes with running water
2. Notify the direct PI/supervisor
3. File an [incident report](#) (SU-17)
4. Seek medical attention (as needed) outlined below:

Employees and students exposed to blood and OPIM in work-related situations should seek medical care at SUOHC located at Environmental Safety Facility, 484 Oak Road (Phone: 650-725- 5308).

Students exposed to blood and OPIM in non-work-related situations (e.g., volunteer first aid) should seek medical care at Vaden Health Center, 866 Campus Drive (Phone: 650-498-2336).

Exceptions:

If the exposure incident occurs outside of SUOHC clinic hours (8 AM to 4 PM weekdays, excluding holidays):

- Report to the Stanford Hospital Emergency Department at 1199 Welch Rd, Palo Alto, CA for immediate care
- Report to the SUOHC on the following working day

In the event of a first aid incident involving the presence of blood or OPIM:

All first aid incidents involving the presence of blood or OPIM must be reported to the employer before the end of the work shift during which the first aid incident occurred.

The incident must be reported using the [incident report form \(SU-17\)](#). The report shall include the following information:

- The names of all first aid providers who rendered assistance, regardless of whether personal protective equipment was used and must describe the first aid incident, including time and date
- A determination of whether or not, in addition to the presence of blood or OPIM, an exposure incident occurred. This determination is necessary in order to ensure that proper post-exposure evaluation, prophylaxis and follow-up procedures are made available immediately if there has been an exposure incident
- The incident report shall be recorded on a list of such first aid incidents. It will be readily available to all employees and shall be provided upon request to the Chief of the Division of Occupational Safety and Health of the California Department of Industrial Relations
- Provision for the full HBV vaccination series will be made available as soon as possible, but in no event later than 24 hours, to all unvaccinated first aid providers who have rendered assistance in any situation involving the presence of blood or OPIM regardless of whether or not a specific exposure incident has occurred

6.2 Post-Exposure Evaluations and Follow-Up

6.2.1 Information Provided to the Healthcare Professional

The following information shall be documented and provided to the SUOHC:

- The route(s) of exposure, and the circumstances under which the exposure incident occurred
- A description of the exposed employee's job duties as they relate to the exposure incident
- Results of the source individual's blood testing, if available and applicable
- All medical records relevant to the appropriate treatment of the employee including vaccination status which are the employer's responsibility to maintain

6.2.2 Source Individual Blood Testing

The PI/supervisor shall identify and document the source individual, if applicable, unless the PI/supervisor can establish that identification is infeasible or prohibited by state or local law.

The source individual's blood shall be tested as soon as feasible and after consent is obtained in order to determine HBV, HCV and HIV infectivity. If consent is not obtained, the PI/supervisor shall establish that consent cannot be obtained. When the source individual's consent is not required by law, the source individual's blood, if available, shall be tested and the results documented.

When the source individual is already known to be infected with HBV, HCV or HIV, testing for the source individual's known HBV, HCV or HIV status need not be repeated.

Results of the source individual's testing shall be made available to the exposed employee, and the employee shall be informed of applicable laws and regulations concerning disclosure of the identity and infectious status of the source individual.

The PI/supervisor must contact SUOHC for assistance in the process. SUOHC will provide aid, as needed and appropriate.

6.2.3 Medical Evaluation and Follow-up

SUOHC shall:

- Evaluate exposed employees according to established medical protocols as immediately as feasible
- Collect the exposed employee's blood if consent is provided and test for BBPs as soon as feasible. If the employee consents to baseline blood collection but does not give consent at that time for HIV serologic testing, the sample shall be preserved for at least 90 days. If within 90 days of the exposure incident, the employee elects to have the baseline sample tested, such testing shall be done as soon as feasible
- Provide post-exposure prophylaxis, when medically indicated, as recommended by the U.S. Public Health Service

6.2.4 Healthcare Professional's Written Opinion

Within fifteen (15) days of evaluation completion, SUOHC shall provide a written opinion to the employee informing them of the results of the evaluation, and any recommendations for further evaluation or treatment.

All other findings or diagnoses shall remain confidential and shall not be included in the written report.

Employees may refuse post-exposure evaluation and follow-up from SUOHC and may instead elect to seek a confidential medical evaluation and follow-up from an independent healthcare professional.

6.2.5 Workplace Evaluation and Follow-up

Within twenty-four (24) hours of a potential exposure incident, an [SU-17](#) must be filed.

As part of evaluating exposure incidents, PIs/supervisors shall as soon as feasible:

- Review the hazard control measures and update it to reflect any corrective measures and improvements
- Provide training, as needed, to affected employees and those with similar potential occupational exposures to prevent future exposure incidents

6.3 Requirements for Sharps Injuries

6.3.1 Sharps Injury Report

Within twenty-four (24) hours of a sharps injury occurring, an [SU-17](#) must be filed.

The assigned EH&S incident investigation lead will conduct an incident investigation and fill out a Sharps Injury Log in RedCap.

6.3.2 Sharps Injury Log

EH&S is responsible for:

- Recording each sharps injury on the Sharps Injury Log within fourteen (14) calendar days of receiving a report of a sharps injury
- Retaining a copy of the Sharps Injury Log for a minimum of five (5) years

The information in the Sharps Injury Log shall be recorded and maintained in such a manner as to protect the confidentiality of the injured person.

EH&S shall perform an annual review of the Sharps Injury Log and document it in the Sharps Injury Annual Report. The review will include, but not be limited to the following:

- Area/Department involved
- Type/model/brand/frequency of use of sharp
- Description of incident

Any identified trend or concern may be further evaluated by a group of representatives from EH&S, SUOHC, Office of Risk Management, and other Stanford University employees as appropriate.

7. Recordkeeping

7.1 Medical Records

SUOHC shall maintain an accurate employee medical record of each employee with occupational exposure in accordance with *California Code of Regulations, Title 8, Section 3204* (8 CCR 3204). SUOHC is responsible for maintaining this record. This record shall include:

- The name and social security number of the employee
- A copy of the employee's HBV vaccination status, including the dates of any HBV vaccinations and any medical records relative to the employee's ability to receive vaccination as required by 8 CCR 5193(f)(2)
- A copy of all results of examinations, medical testing, and follow-up procedures as required by 8 CCR 5193(f)(3)
- A copy of the healthcare professional's written opinion as required by 8 CCR 5193(f)(5)
- A copy of the information provided to the healthcare professional as required by 8 CCR 5193(f)(4)(B)2, 3, and 4

Employee medical records required by 8 CCR 5193 shall be kept confidential and not disclosed or reported without the employee's express written consent to any person within or outside the workplace except as required by 8 CCR 5193 or as may be required by law.

The above-mentioned records shall be maintained for at least the duration of employment plus thirty (30) years in accordance with 8 CCR 3204.

7.2 Training Records

Bloodborne pathogen training course records will be maintained in the Stanford Learning Management System (STARS).

The training records include:

- 1) The dates of the training sessions;
- 2) The contents or a summary of the training sessions.

If an in-person training is requested, the records will include:

- 3) The names and qualifications of persons conducting the training; and

- 4) The names and job titles of all persons attending the training sessions.

Training records shall be maintained for 3 years from the date on which the training occurred.

7.3 Sharps Injury Log

The Sharps Injury Log shall be maintained 5 years from the date the exposure incident occurred.

7.4 Availability

Stanford University shall ensure that all records required to be maintained by this section shall be made available upon request to the Chief of the Division of Occupational Safety and Health of the California Department of Industrial Relations or designated representative ("Chief") and NIOSH for examination and copying.

Employee training records required by this subsection shall be provided upon request for examination and copying to employees, to employee representatives, to the Chief, and to NIOSH.

Employee medical records required by this standard shall be provided upon request for examination and copying to the subject employee, to anyone having written consent of the subject employee, to the Chief, and to NIOSH.

The sharps injury log required by this standard shall be provided upon request for examination and copying employees, to employee representatives, the Chief, the Department of Health Services, and to NIOSH.

7.5 Transfer of Records

The employer shall comply with the requirements involving transfer of records.

If the employer ceases to do business and there is no successor employer to receive and retain the records for the prescribed period, the employer shall notify NIOSH at least three months prior to their disposal and transmit them to NIOSH, if required by the NIOSH to do so, within that three-month period.

Appendix A: Definitions

“Aerosols” means particles less than 10 microns in diameter, and are not typically visible to the naked eye, can remain airborne for extended periods of time, and may be inhaled.

"Biological Cabinet" means a device enclosed except for necessary exhaust purposes on three sides and top and bottom, designed to draw air inward by means of mechanical ventilation, operated with insertion of only the hands and arms of the user, and in which virulent pathogens are used. Biological cabinets are classified as:

1. Class I: A ventilated cabinet for personnel protection with an unrecirculated inward airflow away from the operator and high-efficiency particulate air (HEPA) filtered exhaust air for environmental protection.
2. Class II: A ventilated cabinet for personnel, product, and environmental protection having an open front with inward airflow for personnel protection, HEPA filtered laminar airflow for product protection, and HEPA filtered exhaust air for environmental protection.
3. Class III: A total enclosed, ventilated cabinet of gas-tight construction. Operations in the cabinet are conducted through attached protective gloves.

"Blood" means human blood, human blood components, and products made from human blood.

"Bloodborne Pathogens" means pathogenic microorganisms that are present in human blood and can cause disease in humans. These pathogens include, but are not limited to, hepatitis B virus, hepatitis C virus and human immunodeficiency virus.

"Clinical Laboratory" means a workplace where diagnostic or other screening procedures are performed on blood or other potentially infectious materials.

"Contaminated" means the presence or the reasonably anticipated presence of blood or other potentially infectious materials on a surface or in or on an item.

"Contaminated Laundry" means laundry which has been soiled with blood or other potentially infectious materials or may contain sharps.

"Decontamination" means the use of physical or chemical means to remove, inactivate, or destroy bloodborne pathogens on a surface or item to the point where they are no longer capable of transmitting infectious particles and the surface or item is rendered safe for

handling, use, or disposal. Decontamination includes procedures regulated by California Health and Safety Code Section 118275⁴.

"Engineering Controls" means controls (e.g., sharps disposal containers and needleless systems) that isolate or remove the bloodborne pathogens hazard from the workplace.

"Engineered Sharps Injury Protection" means either:

1. A physical attribute built into a needle device used for withdrawing body fluids, accessing a vein or artery, or administering medications or other fluids, which effectively reduces the risk of an exposure incident by a mechanism such as barrier creation, blunting, encapsulation, withdrawal or other effective mechanisms; or
2. A physical attribute built into any other type of needle device, or into a non-needle sharp, which effectively reduces the risk of an exposure incident.

"Exposure Incident" means a specific eye, mouth, other mucous membrane, non- intact skin, or parenteral contact with blood or other potentially infectious materials that results from the performance of an employee's duties.

"Handwashing Facilities" means a facility providing an adequate supply of running potable water, soap and single use towels or hot air-drying machines.

"HBV" means Hepatitis B virus

"HCV" means Hepatitis C virus.

"HIV" means human immunodeficiency virus.

"Licensed Healthcare Professional" is a person whose licensed scope of practice includes an activity which this section requires to be performed by a licensed healthcare professional.

"Medical Waste" according to California Health and Safety Code Sections 117600 through 118360⁵, commonly known as the "California Medical Waste Management Act", includes but is not limited to the following:

- Human or animal specimens or infectious cultures;
- Sharps, including needles and syringes (clean or dirty);
- Cultures and stocks of infectious agents;
- Wastes from the production of bacteria, viruses, or the use of spores, discarded live and attenuated vaccines, and culture dishes and devices used to transfer, inoculate, and mix cultures;

- Animal parts, tissues, fluids, or carcasses suspected by the attending veterinarian of being contaminated with infectious agents contagious to humans; and
- Waste which contains recognizable blood, fluid blood products, containers or equipment containing blood, or blood from animals known to be infected with diseases which are communicable to humans.

"Needle" or "Needle Device" means a needle of any type, including, but not limited to, solid and hollow- bore needles.

"Needleless System" means a device that does not utilize needles for:

1. The withdrawal of body fluids after initial venous or arterial access is established;
2. The administration of medication or fluids; and
3. Any other procedure involving the potential for an exposure incident.

"NIOSH" means the Director of the National Institute for Occupational Safety and Health, U.S. Department of Health and Human Services, or designated representative.

"Occupational Exposure" means reasonably anticipated skin, eye, mucous membrane, or parenteral contact with blood or other potentially infectious materials that may result from the performance of an employee's duties.

"One-Hand Technique" means a procedure wherein the needle of a reusable syringe is capped in a sterile manner during use. The technique employed shall require the use of only the hand holding the syringe so that the free hand is not exposed to the uncapped needle.

"OPIM" means other potentially infectious materials, which consist of:

1. The following human body fluids: semen, vaginal secretions, cerebrospinal fluid, synovial fluid, pleural fluid, pericardial fluid, peritoneal fluid, amniotic fluid, saliva in dental procedures, any other body fluid that is visibly contaminated with blood such as saliva or vomitus, and all body fluids in situations where it is difficult or impossible to differentiate between body fluids such as emergency response;
2. Any unfixed tissue or organ (other than intact skin) from a human (living or dead); and
3. Any of the following, if known or reasonably likely to contain or be infected with HIV, HBV, or HCV:
 - a. Cell, tissue, or organ cultures from humans or experimental animals;
 - b. Blood, organs, or other tissues from experimental animals; or

- c. Culture medium or other solutions.

"Parenteral Contact" means piercing mucous membranes or the skin barrier through such events as needlesticks, human bites, cuts, and abrasions.

"Personal Protective Equipment" is specialized clothing or equipment worn or used by an employee for protection against a hazard. General work clothes (e.g., uniforms, pants, shirts, or blouses) not intended to function as protection against a hazard are not considered to be personal protective equipment.

"Production Facility" means a facility engaged in industrial-scale, large-volume or high concentration production of HIV, HBV or HCV.

"Regulated Waste" means waste that is any of the following:

1. Liquid or semi-liquid blood or OPIM;
 - a. Contaminated items that contain liquid or semi-liquid blood, or are caked with dried blood or OPIM; and
 - b. Are capable of releasing these materials when handled or compressed
2. Contaminated sharps.
3. Pathological and microbiological wastes containing blood or OPIM.
4. Regulated Waste includes "Medical Waste".

"Research Laboratory" means a laboratory producing or using research-laboratory-scale amounts of blood or OPIM with or potentially containing HIV, HBV or HCV. Research laboratories may use or handle large quantities of blood or OPIMs, but not in the volume found in production facilities.

"Sharp" means any object used or encountered in the industries covered by 8 CCR 5193 Subsection (a) that can be reasonably anticipated to penetrate the skin or any other part of the body, and to result in an exposure incident, including, but not limited to, needle devices, scalpels, lancets, broken glass, broken capillary tubes, exposed ends of dental wires and dental knives, drills and burs.

"Sharps Injury" means any injury caused by a sharp, including, but not limited to, cuts, abrasions, or needlesticks.

"Sharps Injury Log" means a written or electronic record satisfying the requirements of 8 CCR5193 Subsection (c)(2).

"Sharps Injury Annual Report" means a Stanford University written document evaluating Sharps Injury Logs collected on an annual basis.

"Source Individual" means any individual, living or dead, whose blood or OPIM may be a source of occupational exposure to the employee. Examples include, but are not limited to, hospital and clinical patients; clients in institutions for the developmentally disabled; trauma victims; clients of drug and alcohol treatment facilities; residents of hospices and nursing homes; human remains; and individuals who donate or sell blood or blood components.

"Universal Precautions" is an approach to infection control. According to the concept of Universal Precautions, all human blood and certain human body fluids are treated as if known to be infectious for HBV, HCV, HIV, and/or other bloodborne pathogens.

"Work Practice Controls" means controls that reduce the likelihood of exposure by defining the manner in which a task is performed (e.g., prohibiting recapping of needles by a two-handed technique and use of patient-handling techniques).

Appendix B: Non-safety Sharps Documentation

Use this form to document the rationale for using non-safety sharps for patient safety

Employees are NOT required to use engineering controls (i.e., needleless systems, needle devices, or non- needle sharps) if a licensed healthcare professional:

- Determines that the new control will jeopardize the patient's safety or the success of a medical, dental, or nursing procedure
- Is directly involved in the patient's care
- Exercises reasonable clinical judgment

If this exception applies, the healthcare provider is responsible for completing the form below and submit to the Stanford University Biosafety Officer at biosafety-officer@lists.stanford.edu.

Patient Safety Determinations for Exceptions to the Use of Engineering Controls (add additional rows as necessary)

Control Under Consideration	Name of Licensed Healthcare Professional Making Determination	Department/Unit	Date of the Determination	Reasons for the Applicability of this Exception with this Patient
Comments:				

Appendix C: Hepatitis B Declaration Form Statement

HEPATITIS B VACCINE DECLARATION FORM

Please submit this completed form to the address or fax below

Name	SU/Student ID # (8 digits)	Email	
Daytime/work phone	Job Title	Department	Supervisor/PI Name
Stanford Work Location (check all that apply)		Employment Status (select applicable box)	
☐ SHC/LPCH	☐ Spectrum outpatient site	☐ SU Faculty/Staff	☐ SU Employed Student
☐ Stanford Blood Center	☐ Vaden Student Health	☐ Medical Student*	☐ Undergraduate
☐ SU Research Lab, APB/APLAC Protocol #: _____		☐ Student Researcher*	☐ Graduate
☐ Other: _____		☐ Volunteer/Visitor*	☐ Postdoc

** Volunteers and students not engaged in employment or research activity may not be eligible for free vaccination.*

STEP 1, SUPERVISOR/PI:	PLEASE COMPLETE TO CONFIRM EMPLOYEE HEPATITIS B VACCINATION ELIGIBILITY
"I certify that the above-named employee shall be offered Hepatitis B vaccination based upon potential workplace exposure to Bloodborne Pathogens" as listed on the next page .	
Name	Phone:
Signature	Date:

STEP 2, EMPLOYEE:	PLEASE SELECT AND COMPLETE ONE OF THE APPLICABLE SECTIONS BELOW
I. "I WOULD LIKE TO RECEIVE THE HEPATITIS B VACCINE SERIES"	
"Please contact me at the above phone number or email to schedule."	
☐	Employee Signature
	Date:

II. "I DECLINE HEPATITIS B VACCINATION"	
"I understand that due to my occupational exposure to blood or other potentially infectious materials (OPIM) that I may be at risk of acquiring hepatitis B virus (HBV) infection, a serious disease. Although I have been provided the opportunity to be vaccinated with hepatitis B vaccine at no charge* to myself, I decline vaccination at this time. If in the future I continue to have occupational exposure to blood or OPIM at Stanford and wish to be vaccinated, I can receive the Hepatitis B vaccination series at no charge* to myself."	
I have read and understood the above statement and am declining the hepatitis B vaccine series for the following reasons (<i>please check all which apply</i>):	
☐	To the best of my knowledge, I have completed the full Hepatitis B vaccine series (three shots).
☐	I have undergone blood testing showing that I am immune to Hepatitis B.
☐	I will send medical proof of vaccination and/or immunity along with this form
☐	None of the above apply. I do not want to receive Hepatitis B vaccination.
Please fax your Hepatitis B immunization records to SU Occupational Health at 650-725-9218	
Employee Signature	Date:
This 'Declination Statement' for the hepatitis B vaccine is provided in accordance with California Department of Industrial Relations and in accordance with CAL/OSHA. The link to this regulation: http://www.dir.ca.gov/title8/5193a.html	

Occupational Health Services
480 Oak Rd, Stanford, CA 94305 T 650.725.5308 F 650.725.9218

HEPATITIS B VACCINE DECLARATION FORM

To determine if an individual must be included in the Bloodborne Pathogens Program, simply answer the following questions:

*If the answer to **ANY** of the following questions is **YES**, then the worker is considered to be at occupational risk of contracting HBV or other bloodborne pathogens.*

Does the worker ever:

(a) **handle human blood products**, such as whole blood, plasma, serum, platelets, or white cells?

(b) **handle human body fluids** such as semen, cerebrospinal fluid, vaginal secretions, joint fluid, pleural fluid, peritoneal fluid, pericardial fluid, or amniotic fluid?

(c) **work with animals**, such as primates, that are **infected** with hepatitis B or other bloodborne pathogens OR perform tasks where such animals are housed?

(d) **handle unfixed human tissue or organs**? (Tissues and organs soaked in chemical preservatives such as alcohol or formaldehyde are "fixed")

(e) **work with** hepatitis B virus or other bloodborne pathogens or with preparations, such as liquid solutions or powders containing the hepatitis B virus?

(f) **handle blood, blood products, body fluids or unfixed tissues or organs of animals infected** with the hepatitis B virus or other bloodborne pathogens?

(g) **handle sharp instruments** such as knives, needles, scalpels, or scissors which have been used by others working with human blood or other potentially infectious materials to include human organs, tissue or body fluids OR used by others working with similar body parts and fluids from **animals infected** with the hepatitis B virus or other bloodborne pathogens?

(h) **enter areas where other individuals work** with human or animal blood, body fluid, tissues or organs which are **infected with** the hepatitis B virus or other bloodborne pathogens AND perform tasks where these body substances may come into **contact** with the laboratory worker's unbroken skin, broken skin, or mucous membranes?

(i) **perform tasks** which may potentially result in the lab worker's exposed skin or mucous membranes coming in **contact with** human or animal blood, body fluids, organs, or tissues which are **infected with** the hepatitis B virus or other bloodborne pathogens?